Nikki.

FREE GUIDE · CLINIC OWNERS & MANAGERS

Prior Auth & *Claim Denial* Playbook

Step-by-step workflows, a prevention table for the most common denial reasons, and the tracker I'd set up on day one.

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THE IDEA

Denials are cheaper to prevent than to appeal.

Managing insurance verification, prior authorizations, CPT/ICD-10 coding and billing at Tufts Medicine, we reduced claim denials by 20%. The biggest wins weren't clever appeals — they were catching problems before the claim ever went out.

The six-stage workflow

1. **Scheduling intake.** Capture payer, plan, member ID, referring provider and reason for visit at booking. Missing info = the visit isn't confirmed yet.
2. **Eligibility check (2–3 business days out).** Verify active coverage, benefits, copay/deductible and whether the service needs authorization.
3. **Authorization.** Submit with complete clinical documentation; log the reference number, approved units/dates and expiry.
4. **Day-before audit.** Pull tomorrow's schedule: any visit without verified eligibility or a valid auth is flagged and the patient is called.
5. **Same-day coding & scrubbing.** Close the encounter, match codes to documentation, attach the auth number, scrub, submit.
6. **Denial management.** Log every denial by reason code, fix, resubmit or appeal within your target window, and feed the root cause back to step 1–5.

Rule of thumb: every denial reason should map to the stage that could have prevented it. If it keeps happening, that stage needs an SOP update, a checklist or a training refresh.

PREVENTION TABLE

Common denial reasons — and where to stop them

Denial reason	Usually caused by	Prevent it at	Prevention step
Eligibility / coverage terminated	Coverage changed since last visit	Stage 2	Re-verify every visit; re-check at new year and plan changes.
No authorization on file	Auth list out of date, request missed	Stage 3–4	Payer/service auth list; day-before audit of pending auths.
Auth expired / units exceeded	Visits continue past approved dates/units	Stage 3	Log expiry and units; alert 2 weeks before they run out.
Missing or invalid demographics	Typos, old address, wrong subscriber	Stage 1 / check-in	Read back details; scan ID and card every visit.
Coding mismatch / missing modifier	Codes don't match documentation	Stage 5	Same-day coding; cheat sheet of frequent pairs; scrubber edits.
Medical necessity	Diagnosis doesn't support the service	Stage 3 / 5	Attach supporting documentation; confirm diagnosis-to-procedure fit.
Duplicate claim	Resubmitting instead of correcting	Stage 6	Check claim status first; use corrected-claim process.
Timely filing	Claims or appeals sent late	Stage 5–6	Track payer deadlines; work denials within set days.

Exact rules, codes and deadlines vary by payer and contract — keep a one-page summary per major payer.

SCRIPT · CALLING A PATIENT ABOUT A MISSING AUTHORIZATION

"Hi [name], this is [you] from [clinic] about your visit on [date]. Your insurance needs an approval before this visit and we're still waiting on it. We've followed up today. Would you like to keep the appointment and we'll update you by [time], or move it to [option]? Either way, there's nothing you need to do right now."

RUN IT

Cadence, tracker and templates

Daily (15 min)

- Day-before audit: eligibility + auth for tomorrow
- Open encounters from yesterday
- New denials logged and assigned

Weekly (30 min)

- Denial rate and top 3 reasons
- Auths expiring in the next 14 days
- Aging: anything past your target window

Authorization & denial tracker — columns to set up

Column	What goes in it
Patient / MRN · Visit date	Who and when
Payer · Plan · Service / CPT	What's being authorized or billed
Auth # · Approved dates & units · Expiry	So nothing runs out silently
Status · Owner · Next action · Due date	Every row has one person and one next step
Denial reason code · Root-cause stage (1–6)	Turns denials into process fixes
Resolution · Amount recovered · Date closed	Shows the value of the work

TEMPLATE · APPEAL LETTER (OUTLINE)

Date · Payer appeals address · Patient name, DOB, member ID · Claim # and date of service
Re: Request for reconsideration of denied claim [#]
1. What was denied and the reason code given.
2. Why the service was appropriate and covered (plan language, auth #, medical necessity).
3. Supporting documents enclosed (notes, auth approval, corrected claim).
4. The specific action requested (reprocess and pay).
Contact name, phone and signature.

NEXT STEP

Drowning in denials? *Let's find the leak.*

Bring your top denial reasons to a free intro call and I'll help you map them to the step that's causing them — or book a working session to set up the tracker and cadence together.

Session	Length	Price	Best for
Intro call	15 min	Free	Say hello and see if we're a fit.
Mentoring chat	30 min	Free	Career questions, MHA advice or a resume review.
Working session	60 min	On request	A focused session on your clinic or project.

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About Nikki. Nikhila (Nikki) Mamanduri is a Healthcare Operations Manager at Medi-Weightloss® Thousand Oaks with an MHA from the University of New Haven (3.79 GPA) and PMI's CAPM®. She has worked across medical devices (Relisys), dialysis (U.S. Renal Care), academic medicine (Tufts Medicine) and clinic operations, and believes better systems create stronger teams — and a better experience for every patient.

This guide shares general operational practices for educational purposes. It isn't legal, billing-compliance, immigration or medical advice — check payer rules, your organization's policies and the relevant authorities for your situation.